

D15 — MICU Progress Note

Dr. E. Park, MICU Fellow | MICU | 02/23/2026 07:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

MICU PROGRESS NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156
Date: 02/23/2026 07:00 | ICU Day 2 | Provider: Dr. E. Park, MICU Fellow

OVERNIGHT:
Required norepinephrine for MAP < 65 – weaned off within 10 hours of MICU admission. Received 2 additional units pRBCs (4 total this admission). No further melena or hematemesis.

VITALS: HR 82, BP 108/62 (off pressors), RR 16, SpO2 95% on 2L NC
LABS (AM):
Na 134 | K 5.2 (↑↑) | Cl 104 | CO2 22 | BUN 48 | Cr 2.3 (↑↑ – AKI peak) | Glucose 310
Hgb 8.2 (stable post-transfusion) | Plt 178

ASSESSMENT & PLAN:

- GI hemorrhage – hemodynamically stable off pressors. Hgb stable 8.2. No evidence of re-bleeding. Continue pantoprazole drip. Serial Hgb Q8H (de-escalating from Q6H).
- AKI on CKD – Cr 2.3, up from baseline 1.5. Likely pre-renal from hemorrhagic shock + volume depletion. Urine output improving with resuscitation. Hold nephrotoxic agents. Plan to restart home medications as renal function improves.
- Hyperkalemia – K+ 5.2.
 - STOP KCl supplementation immediately (was started yesterday when K+ was 3.5 pre-bleed – order should have been discontinued)
 - Hold spironolactone (AKI + hyperkalemia)
 - Hold lisinopril (AKI + hypotension)
 - Carvedilol: reduce to 12.5 mg BID (hypotension, SBP running 100-110)
 - Glipizide: HOLD (poor PO intake, NPO intermittently – hypoglycemia risk)
 - Allopurinol: HOLD (AKI)
- CHF – diuresis interrupted. Will resume when euvolemic and renal function stabilizing.
- COPD – improving. Continue nebs. Prednisone 30 mg today (taper step-down).

6. DM – glucose 310, steroid effect + stress. Increase insulin sliding scale coverage.

7. Constipation – worsening with immobility + reduced PO + blood products. Continue docusate, senna scheduled, MiraLAX.

Electronically signed by: Dr. E. Park, MD

Date: 02/23/2026 07:45

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